CASHLESS BUT NOT SPOTLESS

THE 'SWASTHYA SATHI' HEALTH COVER SCHEME HAD BEEN MAMATA BANERJEE'S ANSWER TO THE CENTRE'S AYUSHMAN BHARAT ROLLOUT. A CAG REPORT NOW 'EXPOSES' ITS INEFFICIENCIES

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For almost a decade, the Swasthya Sathi was West Bengal's flagship health insurance scheme, the centrepiece of Trinamool Congress (TMC) chief Mamata Banerjee's welfare politics. The cashless model, launched on December 30, 2016, had a sweeping mandate—offer a Rs 5 lakh annual cover to all permanent residents of the state (except those covered under other public health schemes such as state employees). The TMC regime even chose to walk away from the central healthcare Ayushman Bharat alliance, arguing that its own programme was superior to it. But now, a Comptroller and Auditor General (CAG) audit has cast a shadow over that legacy.

The report on the Swasthya Sathi scheme was only tabled in the assembly after the BJP came to power. The audit—which cover the period between 2018 and 2023 when Bengal spent Rs 6,882 crore on the scheme—points to a few alleged financial irregularities. The findings include, among other things, excess expenditure/losses on enrolment and smart card distribution (Rs 47 crore), unrecovered penalties from insurance firms (Rs 31.3 crore), refunds that insurers have yet to pony up (Rs 9.8 crore), and excess reimbursements to private hospitals with inflated grades (Rs 14.5 crore). The total financial impact from the lapses is estimated to be around Rs 117.6 crore (about 1.7 per cent of total spend), according to the report.

A red flag raised by the CAG was beneficiary enrolment exceeding ration card-holding families by 10 per cent; as of March 2023, 24.3 million families had been enrolled for Swasthya Sathi while only 22.1 million enrolled in the public distribution system (PDS) system. But then, this was a universal insurance scheme where any Bengal resident could enrol, not just those who qualified for PDS rations.

INFLATED GRADINGS

Another significant observation in the audit concerns how hospitals were graded, and how it influenced reimbursements. The CAG found that only 2,605 healthcare providers had been empanelled against an assessed requirement of 3,038 hospitals, limiting the treatment options. Also, all 27 government superspeciality hospitals empanelled under the scheme had been classified below Grade A despite possessing tertiary-care infrastructure. Since package rates depended on grading, lower clas-